RESEARCH AND EVIDENCE AUDIT

The Small Changes Manual

Checked 25 October 2026. Targeted evidence review, not a registered systematic review. Guidelines, reviews and original human studies were preferred. Publicly retrievable abstracts, source pages and available full text were used; some full-text endpoints were blocked. No claim of exhaustive database coverage or clinical peer review is made.

Habit and cue based repetition / Mixed

Sources [1] [2]. Human interventions and planning studies support context-linked repetition. Designs and self-reported automaticity vary. No 21-day or 28-day promise.

Pages 4-7; two minutes, identity language and room setup are practical adaptations, not validated universal prescriptions.

Sleep opportunity and body timing / Strong

Sources [3] [4] [7]. Consensus and public guidance support enough sleep and a regular pattern. At least seven hours is adult guidance, not an optimum for every person.

Pages 8-11; sleep need, sleep opportunity and time asleep are distinguished. Shift work and persistent fatigue require individual attention.

Daytime and evening light / Strong for clock effects

Sources [5]. Expert consensus integrates human physiology studies. Exact outdoor minutes cannot be inferred from indoor lighting recommendations.

Page 9; no fixed dose, direct sun gazing or mandatory sunrise exposure. Seasonal, eye and work circumstances qualify advice.

Caffeine and sleep / Strong for disruption

Sources [6]. Systematic review and meta-analysis of human studies. Dose, timing and individual differences limit universal cutoff claims.

Page 10; earlier or smaller caffeine is a personal experiment. No requirement to delay morning coffee.

The Small Changes Manual

Persistent insomnia / Strong

Sources [8]. AASM guideline recommends multicomponent CBT-I and advises against sleep hygiene alone as sole treatment.

Page 11; brief environmental changes are not sold as treatment. No unsupervised sleep restriction.

Activity and strength / Strong

Sources [9]. WHO guideline synthesis: 150-300 moderate or 75-150 vigorous minutes weekly, or combination, plus strengthening major groups at least twice weekly.

Pages 12-14; tiny sessions are starters. Chair rises and wall push-ups alone do not cover all major groups. Adapt for pain, health, access and disability.

Less sedentary time / Strong for broad guidance

Sources [9]. Guidelines support replacing sedentary time with activity. They do not establish one ideal break interval for all adults.

Page 14; cue-based movement is an example. A standing break does not replace broader activity.

Food quality and fluid / Strong for general guidance

Sources [10] [11]. WHO and NHS guidance supports dietary variety and adequate fluid. Needs vary with conditions, climate and activity.

Page 15; no universal large water dose or superfood. Respect allergies, eating disorders and prescribed fluid limits.

The Small Changes Manual

Notifications and focus / Mixed for real life transfer

Sources [12]. Controlled human attention experiment supports a distraction cost. A laboratory task does not establish a fixed loss of work minutes.

Pages 16-17; phone placement and work block length are design experiments. Essential contact and job needs remain available.

Micro-breaks / Mixed

Sources [13]. Meta-analysis supports vigour and fatigue outcomes more clearly than task performance. Study contexts and break activities vary.

Page 17; ten minutes is an adjustable example, not an optimal biological schedule.

Recall and spacing / Strong

Sources [14] [15]. Meta-analyses support retrieval practice and distributed learning. Much evidence comes from students and verbal learning tasks.

Page 18; feedback and correction included. Tomorrow and later this week is illustrative timing, not a tested optimum for every skill.

Breathing for stress / Mixed

Sources [16] [20]. Fincham review: 12 trials and 785 adults for stress; average effect g=-0.35, 95% CI -0.55 to -0.14. Most studies had moderate risk of bias.

Page 19; no numerical promise in book. A one-minute gentle pause is an extrapolation; it is not the full trial intervention. Stop for dizziness or increased anxiety.

The Small Changes Manual

ACT inspired thought and values exercises / Mixed for standalone exercise

Sources [17] [20]. A-Tjak review includes 39 RCTs with 1821 clinical participants. Therapy-level results do not establish the effect of our short phrase or worksheet.

Pages 20,22; not presented as superior to established therapy or as a treatment. Acceptance does not mean tolerating abuse.

Social connection / Strong association; causal dose uncertain

Sources [18]. Meta-analysis of prospective observational studies supports association with survival. Confounding and reverse causation remain possible.

Page 21; no claim that a call extends lifespan or guarantees happiness. Quality and safety matter.

Cold exposure and other optimisation / Early or not established for required use

Sources [19]. Cold-water review reports limited trials, varied protocols and selective outcomes. Broader wellness claims exceed the confidence available.

Page 32; no cold prescription. Excluded stacks, dopamine resets and rigid timing were not individually established as useful additions by this review.

The combined four-week system / Not a validated programme

Sources [1] [2] [3] [4] [5] [6] [7] [8] [9] [10] [11] [12] [13] [14] [15] [16] [17] [18] [19] [20]. Original editorial synthesis. No trial tested this exact book, sequence, daily plan or minimum action set.

Pages 23-31; no guarantee of habit formation, improved health or productivity within four weeks. Review barriers and retain only useful actions.

The Small Changes Manual

Source and editorial checks

All 20 numbered sources are listed with titles, author or organisation, year and source URL in the book. DOI links are used where confirmed; PubMed, PMC or official guidance links are used elsewhere. Retrieval verified the cited records; future uptime and access are not guaranteed.

Influential books are credited on page 32. Full book manuscripts were not available for complete audit. Huberman Protocols was verified via official publisher descriptions as a 2026 publication. Its exact recommendations were not represented as having been exhaustively checked.

Labels describe the stated evidence, not every practical exercise nearby. Strong means consistent substantial evidence or established guidance. Mixed means plausible support with important limitations. Early means insufficient reliable evidence. These are editorial judgements, not GRADE ratings.

Numerical checks: sleep guidance is at least seven hours regularly; activity guidance includes moderate and vigorous alternatives plus strengthening twice weekly. Two-minute habits, five-minute walks, ten-minute focus blocks and 28 days are starter design choices, not evidence-based minimum therapeutic doses.

Contradictions resolved: enough sleep takes precedence over early rising; tiny starts lead toward useful activity amounts; consistent cues allow variable schedules; supportive connection includes boundaries; focus supports necessary contact; emotional acceptance allows practical action and treatment.

Original educational diagrams were drawn deterministically. Alex scenes were generated as one consistent illustration sheet and cropped for layout. Illustrations show examples and do not serve as physiological evidence. The editable edition keeps text and tables native; illustrations and diagram art are embedded images.

Excluded claims: fixed habit deadlines, guaranteed identity transformation, permanent brain rewiring promises, dopamine resets, cold exposure as a necessary health practice, compulsory supplement stacks, universal caffeine timing and one ideal work-break ratio. Exclusion is not proof that every version of an intervention is ineffective.

General scope: this is a selected daily-life handbook, not a complete preventive-health guide. It does not replace medical care, vaccination, screening, medication or support for structural barriers. No named book author or clinician endorsed this manual.

Publication and rights review

Independent educational edition • checked 25 October 2026

Purpose

One Small Shift is an independent educational comic about everyday wellbeing. Educational purpose describes intent. It does not create a blanket copyright exception, and it is not a medical treatment.

Original writing and design

The explanations, fictional Alex story, speech bubbles, page layouts, diagrams and four-week plan were created for this project with AI assistance. Scientific references support the stated claims. Books are credited as influences, not treated as proof.

What is not included

No book covers, scanned pages, publisher artwork, copied signature diagrams, substantial quotations, celebrity likenesses or third-party stock photos are included. The project does not imitate a named artist or claim endorsement by an author, publisher or medical organisation.

Artwork provenance

Twenty-four fictional comic scenes were generated for this project. Their prompts use the project’s own character references and generic visual descriptions. The repository retains a provenance manifest. This records process; it does not guarantee that no third-party similarity or claim could arise.

Limits of the review

Full source books were unavailable for complete comparison. No exhaustive plagiarism corpus, comprehensive image-similarity search, trademark search or jurisdiction-specific legal clearance was performed. No zero-risk copyright guarantee is offered.

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Fonts and dependencies

The webpage uses system fonts. The PDF embeds DejaVu fonts; the applicable licence notice is retained with the source. Python libraries are build dependencies. Their source is not copied into this repository.

Source and health review

Sources were checked 25 October 2026 through a targeted evidence review. Some scientific records were available as abstracts only. This is not a systematic review, a medical peer review or a complete preventive-health guide. Clinical advice must be individualised.

Privacy

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Fair use depends on the particular circumstances

AI copyrightability and human authorship